Weight Management — Medication Quick Reference

Summary of all anti-obesity medications: expected outcomes, key safety points, and what to watch for

ORLISTAT OTC

Orlistat

Xenical® 120mg Rx · Alli® 60mg OTC

Goal: 5–10% weight loss
How: Blocks fat digestion in the GI tract
SE: Oily discharge, flatulence, vitamin A/D/E/K loss
Avoid: Malabsorption, cholestasis, warfarin
Age: ≥12 obesity

PHENTERMINE SCHED IV

Phentermine

Adipex-P® · Lomaira® · Suprenza® · 12-wk max

Goal: 5–10% weight loss
Dose: 15–37.5mg PO qAM · max 37.5mg
SE: ↑HR, ↑BP, dry mouth, insomnia, nervousness
Avoid: CAD, uncontrolled HTN, MAOIs, glaucoma, pregnancy
Age: ≥16 obesity

LIRAGLUTIDE GLP-1 SC DAILY

Liraglutide

Saxenda® obesity · Victoza® diabetes

Goal: 5–10% weight loss
Dose: 0.6→3.0mg SC daily (weekly titration)
SE: Nausea, vomiting, diarrhea (improve with titration)
Avoid: Medullary thyroid CA, MEN2, pancreatitis, pregnancy
Age: ≥12 obesity

NALTREXONE/BUPROPION RX PO

Naltrexone / Bupropion

Contrave® · 8/90mg tablets

Goal: 5–10% weight loss
Dose: Titrate: 1 AM → 2 AM + 2 PM (4 weeks)
SE: Nausea, constipation, headache, insomnia, dry mouth
Avoid: Opioids, seizures, uncontrolled HTN, MAOIs, pregnancy
Age: ≥18 obesity

PHENTERMINE/TOPIRAMATE SCHED IV QAM

Phent/Topiramate ER

Qsymia® · ⚠️ Monthly pregnancy test!

Goal: 7–10% weight loss
Dose: 3.75/23→7.5/46→11.25/69→15/92mg QAM
SE: Paresthesia, dizziness, dry mouth, constipation, ↑HR
Avoid: **Pregnancy (orofacial clefts!), glaucoma, MAOIs, hyperthyroidism**
Age: ≥18 obesity · Never stop abruptly

SEMAGLUTIDE GLP-1 SC WEEKLY

Semaglutide

Wegovy® obesity · Ozempic® diabetes

Goal: ~15% weight loss
Dose: 0.25→0.5→1→1.7→2.4mg SC weekly (4-wk steps)
SE: Nausea, diarrhea, vomiting, constipation, fatigue, GERD
Avoid: Medullary thyroid CA, MEN2, pancreatitis, pregnancy
Age: ≥18 obesity

TIRZEPATIDE GLP-1+GIP ⭐ SC WEEKLY

Tirzepatide — Highest Efficacy

Zepbound® obesity · Mounjaro® diabetes

Goal: 20–25% weight loss
Dose: 2.5→5→7.5→10→12.5→15mg SC weekly (4-wk steps)
SE: Nausea, diarrhea, vomiting, constipation, fatigue
Avoid: Medullary thyroid CA, MEN2, pancreatitis, pregnancy
Age: ≥18 obesity

METFORMIN RX PO · OFF-LABEL

Metformin

Glucophage® · Off-label for weight management

Goal: Modest weight loss · improves insulin resistance
Dose: 500mg PO QD → titrate to 2000mg QD
SE: GI upset; lactic acidosis (rare but serious)
Avoid: eGFR <30, metabolic acidosis, DKA, dehydration
Age: ≥10 for T2DM

OUR APPROACH TO WEIGHT LOSS CARE

📅 Your Follow-Up Schedule

- **Monthly** — first 3 months after starting
- **Quarterly** — ongoing monitoring visits

🏥 Our Multi-Disciplinary Team

🥗 Dietician · 🏋️ Exercise Therapist · 🧠 Psychologist · ⚕️ Board-certified DABOM physician

🔑 Keys to Success

All medications work best combined with reduced-calorie diet, regular activity, and behavioral support. **Every 1% weight loss matters for your health.**

💪 Your Weight Loss Journey — Backed by Science, Supported by Our Team

Obesity is a chronic medical disease requiring long-term management. The right medication for you depends on your health history, other medications, insurance, and personal preferences. Together, we'll find the approach that works best for your unique situation. You don't have to do this alone. 💛 — Dr. Geffrey H. Klein, MD FACOG DABOM · (281) 557-0300 · 400 W Medical Center Blvd, Suite 300, Webster TX 77598